Dacarbazine — metastatic melanoma

The treatment this guide covers This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Dacarbazine — metastatic melanoma · support plan

Active medicines: Dacarbazine

Emetogenic risk: High per National Cancer Control Programme (NCCP) 00464 v3; preventive antiemetic regimen required under local policy

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 Current NCIS V8 non-AC high-risk row; review interactions.
Ondansetron	16 mg orally	Day 1 Review QT risk, electrolytes, constipation and hepatic dosing.
Dexamethasone	12 mg orally	Day 1 Reconcile the complete steroid plan and aprepitant interaction.
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, glucose and interactions.
Aprepitant	80 mg orally once daily	Days 2–3 Check the take-home schedule.
Dexamethasone	8 mg orally once daily	Days 2–4 Non-AC high-risk schedule; do not substitute the AC row.
Metoclopramide	10 mg orally up to three times daily as needed	Breakthrough / subsequent days; duration to be prescribed Caution with olanzapine because of additive extrapyramidal effects; review renal function, contraindications and alternative rescue options.

Additional phase-specific support

- Use an individual approved high-risk antiemetic prescription, with prevention and breakthrough instructions. The source-based medicines below are a reviewable reference, not an automatic prescription; reconcile interactions, steroid exposure, sedation, QT risk and rescue alternatives.
- Dacarbazine has high nausea-and-vomiting risk, so a planned preventive anti-sickness regimen is needed under the local protocol, including clear take-home instructions. The NCCP line that routine premedication is usually unnecessary does not remove this antiemetic requirement. Ask what to take regularly, what is rescue treatment and when to call if it fails.
- Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.
- Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.
- The current HPRA product information advises effective contraception during treatment and for six months after the final dose for women, and during treatment and for three months afterwards for men. Discuss fertility preservation and, when relevant, genetic counselling before treatment. Do not breastfeed during treatment; ask the team about the appropriate interval afterwards.
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Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00464 v3 · p4 · NCCP NCIS medical-oncology antiemetics V8, Table 1 p4, high risk excluding AC

Sources checked 2026-11-06 · SACTCheck v0.84.42 · Regimen-level drafts; prescriber and oncology-pharmacy verification required